



Procedure

Mental Health Medically Cleared for Discharge Procedure

Original endorsement date	December 2025
Date of current endorsement	December 2025
Version	1.00
Functional subgroup	Mental Health
Summary	To identify when mental health patients are medical cleared for discharge from the Mental Health Inpatient unit Services
Procedure owner	EMHS Executive Director Mental Health
Contact	Program Manager, Mental Health Strategy
Review date	December 2028
NSQHS Standards (second edition)	
Impact Statement Declaration Consultation (ISD)	The Aboriginal consultation is reflected in this policy to ensure cultural appropriateness and positive health impacts on Aboriginal people have been considered (ISD Record ID).
Status	Active
Information Classification	OFFICIAL



1. Purpose

To ensure a standardised and consistent approach across EMHS Mental Health Services in identifying when a person admitted to a Mental Health Inpatient unit is deemed Medically Cleared for Discharge (MCFD). This procedure outlines responsibilities, documentation requirements, and processes to support identification and compliance with Department of Health requirements.

2. Background

The introduction of standardised long-stay discharge delay data collection through webPAS fields allows consistent capture of discharge delay reasons for Inpatients who are MCFD across the WA public health system. Identification of these discharge delay reasons supports a system wide approach and management of long stay strategies such as:

- informing operational improvements, planning and delivery
- providing greater visibility of discharge delay reasons and advocacy for improvements and change at a system level
- reducing reliance on manual reporting
- reducing duplicate reporting
- improving consistency of reporting.

3. Definitions

Term	Definition
Medically Cleared for Discharge (MCFD)	An inpatient is determined as medically cleared for discharge when the treating Consultant Psychiatrist, and multidisciplinary team, determines that the person's care needs can be met outside of an inpatient admitted setting, but for the delaying factor remains an inpatient .
Care Type	Care Type is a classification system used for admitted episodes of care. Care type data is used to inform a wide range of management, planning, reporting and financial functions. The care type is allocated on webPAS. For mental health inpatient wards the care type is always "Mental Health".
Mental Health Care Type	Mental Healthcare addresses psychosocial, environmental, and physical functioning. It requires specialist care, a mental health care plan, and recorded National Outcomes and Casemix Collection (NOCC) measures.

Mental Health Phase of Care	Mental Health Phase of Care is a measure of the primary goal of care and is used for activity-based funding. Staff will allocate and change a person's phase of care throughout their admission to align with their current treatment goal. This is completed on PSOLIS.
Episode of Care – Care Type Change form	This form is used in the Mental Health Setting to identify that someone has been deemed MCFD and the discharge delay reason(s).

4. Procedure

4.1. Determining Medically Cleared for Discharge (MCFD)

The multidisciplinary team (MDT) and treating Consultant Psychiatrist will determine if a person is deemed Medically Cleared For Discharge (MCFD). This means identifying that the person's care needs can be met outside of an inpatient admitted setting, but due to the delaying factor (e.g. the availability of suitable accommodation) remains an inpatient.

Inpatients detained under the Mental Health Act 2014, who cannot be discharged from hospital due to clinical care that requires inpatient treatment, should not be classified as MCFD.

4.2. Reasons for Discharge Delay for a MCFD

Discharge delay reasons need to be identified by the MDT and Consultant Psychiatrist. A primary, and if applicable a secondary, reason must be identified from the list in **Appendix 1** and entered on the Episode of Care – Care Type Change form.

5. Documentation Requirements

5.1. Episode of Care – Care Type Change (EOC-CTC) Form

For EMHS Mental Health Inpatient Services the treating Consultant / or delegate is responsible for ensuring the below:

1. The EOC-CTC form is completed with the below information:
 - Date and time that the person was deemed to be MCFD
 - Care Type: Remains Mental Health – NO CARE TYPE CHANGE
 - Treating consultant name
 - Diagnosis
 - “Reason for care type change” section is documented as below:
 - Mental Health – **no care type change**, MCFD status only

- Primary and Secondary reason (where applicable) is indicated
 - Consultant or delegate signature
2. Clinical documentation is completed in the health care record, inclusive of decision to be deemed as medically cleared for discharge, discharge delay reasons, and plan.
 3. A review Phase of Care and NOCC is completed in PSOLIS, changing the Phase of Care to Consolidating Gain.

5.2. WebPAS Data Capture

The ward clerk will enter the EOC-CTC form into webPAS. The information required to be entered will be the MCFD reasons and date identified on the EOC-CTC form, into webPAS.

The ward clerk will send the EOC form to be scanned into the DMR as a matter of priority.

Patient Systems Support (RPBG / AKG) will monitor daily that there have been no changes of care type completed on webPAS for all MH wards.

6. Reversal of MCFD Status

If a person has been identified as MCFD, this must be regularly reviewed as part of the clinical review process to ensure they still meet criteria and the MCFD reason is still valid. If the person's medical status changes and they are no longer MCFD, as decided by the Consultant Psychiatrist and MDT, the following process is to be followed:

6.1. Episode of Care – Care Type Change (EOC-CTC) Form

For EMHS Mental Health Inpatient Services the Nurse Unit Manager / or delegate is responsible for ensuring the below:

1. The EOC-CTC form is completed, indicating the date the person is no longer MCFD.
2. The EOC-CTC form is provided to the ward clerk, for the MCFD status to be reversed in webPAS.
3. Clinical documentation in the healthcare record indicating the rationale for the person to be no longer being MCFD and the current plan.
4. A review Phase of Care and NOCC is completed in PSOLIS, changing the Phase of Care to no longer being Consolidating Gain.

7. Compliance monitoring and legislative penalties

7.1. Compliance monitoring

Each site Coordinator of Nursing supported by the mental health long stay coordinator will monitor compliance through the DoH Medically Cleared for Discharge Dashboard.

- [Medically Cleared For Discharge - Power BI](#)

EMHS Mental Health Executive Director is responsible for ensuring staff in their area/department are made aware of and comply with this procedure, and that compliance, risks and any training requirements are managed.

7.2. Legislative obligations

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

8. Legislation

- Bullet
- Bullet

9. Additional references

9.1. WA Health policies

- [Admitted-Patient-Activity-Data-Business-Rules.pdf](#)

9.2. EMHS policies

- [EMHS Clinical Documentation](#)

10. Appendix 1 – Reasons for Discharge Delay for a MCFD

1. Awaiting for NDIS funding approval
2. Awaiting NDIS funded supports to engage
3. Availability of suitable residential aged care place (including Transition Care Program and respite)
4. Availability of suitable accommodation option (excluding residential aged care)
5. Awaiting specialist clinical community services
6. Awaiting community mental health supports / transition place
7. Awaiting State Administrative Tribunal (SAT) guardianship and/or administration processes
8. Awaiting Child Protection / foster placement outcome or child-related concern
9. Patient / carer / decision maker has no financial means/is unwilling to pay fees with proposed discharge option
10. Patient / carer / decision maker not consenting to proposed discharge option
11. Limited supports or equipment available in the community to meet ongoing complex medical, mental health and / or cognitive care needs (to be selected as a secondary reason only)
12. Awaiting aged care services in the home (CHSP / HCPP)
13. Need for patient / carer / parent / provider education or training